Medical Records and Clinical Documentation

MS-MR-55 — Course handbook, module map and assessment structure.

Module	Handbook
Study time	20 hours total
Audience	Medical records staff and officers, coders, billing and documentation teams, doctors, nurses, allied health, quality and administration
Assessment	Knowledge check in every module, a 50-question scored bank per module, and a 100-question final assessment
Standards basis	NABH 6th Edition (IMS, AAC, COP, MOM, PRE, PSQ, ROM); IMC Regulations 2002; Consumer Protection Act 2019; DPDP Act 2023; Clinical Establishments Act; MTP Act; HIV/AIDS Act 2017; Mental Healthcare Act 2017; EHR Standards for India

This course is built for the people who actually write, handle, complete, code, release and audit hospital records in India. It assumes no prior training in medical records and takes the subject from what a record is in law through to the governance structure that keeps it working. Every module is written to be used on the ward and in the department rather than memorised for an examination.

0.1 Who this course is for

Role	What this course gives them
Medical records staff and officers	The full operational picture: assembly, analysis, deficiency chase, numbering, filing, retrieval, retention, destruction, release of information and medico-legal handling
Coders and billing staff	How documentation becomes a code, the phrases that make a record uncodeable, compliant querying, and the documentation grounds on which claims are rejected
Doctors	What a defensible entry contains, consent and refusal documentation, operative and procedure notes, discharge summaries, death certification, and why coders query
Nurses	The nursing record set, risk screens, flow sheets, escalation documentation, handover records, medication administration and transfusion documentation
Quality and accreditation teams	The NABH standards tested through the file, documentation audit design, indicators, and the governance structure
Administration and management	Where the hospital’s documentation risk sits, what it costs in four currencies, and what management must resource and enforce

0.2 The twelve modules

#	Module	What it covers
1	Purpose, Ownership and Legal Standing	What the record is for, what counts as part of it, who owns the file and the information, the Indian legal framework, the three-year and seventy-two hour rules, the record as evidence, and the NABH standards tested through it
2	Structure and Contents	Assembly order, mandatory contents, page and entry identification, outpatient, emergency and day-care record sets, forms control and version management, and the deficiency checklist
3	Documentation Principles	The ten principles, objectivity, legibility, error correction, late entries and addenda, prohibited abbreviations, countersignature and supervision, and where documentation predictably fails
4	Clinical Notes in Practice	The first twenty-four hours, initial assessments, risk screens, progress notes, reassessment intervals, the nursing record set, escalation documentation, handover and transfer

#	Module	What it covers
5	Medication, Orders and Treatment	The anatomy of a medication order, allergy documentation, verbal order read-back, high-alert medications, administration and omission coding, reconciliation at three points, transfusion, and adverse drug reactions
6	Consent, Refusal and Choice	Elements of valid consent, capacity, information disclosure, who may consent in each situation, consents with their own statutes, refusal and DAMA documentation, and consent audit
7	Investigations and the Perioperative Record	Requisitions, critical result escalation, the nine perioperative documents, the operative note, implant traceability, anaesthesia and sedation, procedures outside theatre, and the specimen chain
8	Discharge, Transfer and Death	Discharge summary contents, pending results, the five exit routes, DAMA, absconding, transfer-out summaries, death documentation, cause-of-death structure, medico-legal deaths and body handover
9	Coding and Classification	From bedside to bill, reading an ICD-10 code, the four diagnosis types, phrases that make a record uncodeable, compliant querying, under- and over-coding, CDI, and claim rejection grounds
10	Medical Records Department Operations	Record movement with time standards, numbering systems and the master patient index, terminal digit filing, issue and tracking, deficiency chase, retention, destruction and department indicators
11	Confidentiality, Access and Medico-Legal	The basis of the duty, who may be given the record, the release workflow, police and court requests, categories with stricter rules, medico-legal cases and injury reports, and data breaches
12	Electronic Records, Audit and Governance	EMR failure modes and controls, admissibility and audit trails, downtime, interoperability standards, the documentation audit cycle, indicators, and the governance structure

0.3 How the course is assessed

Each module ends with a short knowledge check of applied, scenario-based questions with model answers, intended for discussion rather than scoring. Scored assessment is delivered separately as JSON question banks for loading into the hospital’s learning management system.

Assessment	Questions	Difficulty profile	Pass mark
Module question bank (one per module)	50 single-best-answer questions, 600 in total	334 moderate and 266 high across the twelve banks: application, discrimination and judgment rather than recall	70 per cent
Final assessment	100 single-best-answer questions drawn across all twelve modules	29 moderate and 71 high, weighted towards cross-module application and synthesis	70 per cent

Every bank item carries the correct answer, a rationale explaining why the other options are wrong, the module and topic it maps to, and the standard or statute it rests on, so that the banks double as a revision resource and as evidence of assessment content for accreditation purposes.

> **KEY POINT**
> The question banks are supplied as JSON files alongside the module PDFs, in a schema designed for direct import into a learning management system. A combined file containing all banks is also included.

The question bank file format

Each file carries the course identifiers, the bank type, the question count, the difficulty profile and the pass mark, followed by an array of questions. Every question carries a stable identifier, the question type, its difficulty, the topic

it tests, the stem, four lettered options, the keyed answer, a rationale and the module section it rests on.

Field	What it holds
id	A stable identifier such as MS-MR-55-M04-Q017 or MS-MR-55-FINAL-Q062, usable as the primary key on import
type	single_best_answer throughout; one correct option out of four
difficulty	moderate or high, allowing a bank to be filtered or weighted at delivery
topic	The sub-topic within the module, so that a learner's weak areas can be identified from their result
stem	The question as the learner sees it
options	Four options keyed A to D; the answer key is distributed evenly across the four letters
answer	The letter of the correct option
rationale	Why the keyed option is correct, and where useful why the others are not; suitable for release as feedback after scoring
reference	The module section the item rests on, so a learner can be sent back to the right page
marks	One mark per question; the bank carries shuffle_questions and shuffle_options flags set true and negative_marking set false

0.4 Standards and statutes this course is built on

Source	Where it is used
NABH 6th Edition — IMS chapter	Throughout; record contents, authentication, abbreviations, retention, destruction, confidentiality and record review
NABH 6th Edition — AAC chapter	Modules 4 and 8; assessment, reassessment, continuity, transfer and discharge
NABH 6th Edition — COP chapter	Modules 5 and 7; high-risk care, surgical and anaesthesia documentation, transfusion, end-of-life care
NABH 6th Edition — MOM chapter	Module 5; prescription, administration, verbal orders, reconciliation, high-alert medications
NABH 6th Edition — PRE chapter	Modules 6 and 11; consent, patient rights, privacy, access to records
NABH 6th Edition — PSQ and ROM chapters	Module 12; documentation audit, indicators and governance
IMC (Professional Conduct, Etiquette and Ethics) Regulations 2002	Modules 1, 10 and 11; retention, supply of records within seventy-two hours, confidentiality
Consumer Protection Act 2019	Module 1; the record as evidence of service
Indian evidence law (Bharatiya Sakshya Adhiniyam 2023)	Modules 1 and 12; admissibility of electronic records
Digital Personal Data Protection Act 2023	Modules 1, 10, 11 and 12; health data, security, retention and breach
MTP Act, HIV/AIDS Act 2017, Mental Healthcare Act 2017, THOTA, PCPNDT Act	Modules 6 and 11; statutory consents and restricted categories
Registration of Births and Deaths Act and MCCD requirements	Module 8; death certification and cause-of-death structure
EHR Standards for India and ABDM	Module 12; terminologies, exchange and consent-based sharing

0.5 How to use this course in a hospital

1. Assign all twelve modules to medical records staff, coders and quality teams.
2. Assign Modules 1, 3, 4, 5, 6, 7, 8 and 9 to doctors, and Modules 1, 2, 3, 4, 5, 6 and 8 to nurses, as the minimum clinically relevant set.
3. Run each module's knowledge check as a departmental discussion rather than a written test; the questions are written for that purpose.
4. Use the scored banks through the learning management system, with the final assessment as the certification gate.
5. Pair the course with a baseline documentation audit, so that the training has a measured starting point and the re-audit can show the effect.
6. Review the hospital's own policies against the content as you go; where the course describes a control the hospital does not have, that is a finding worth recording.

0.6 A note on scope and local policy

This course states general positions under Indian law, professional regulation and accreditation standards as at the date of writing. Retention periods, consent formats, abbreviation lists, critical value lists, deficiency standards and release-of-information authority vary between hospitals and between states, and in every case the hospital's own approved policy applies. Where the course and the hospital's policy differ, follow the policy and raise the difference with the medical records committee — which is, in itself, the behaviour this course is trying to produce.

> **CAUTION**
> Nothing in this course is legal advice. Where a specific situation carries legal consequence — a summons, a police requisition for a non-medico-legal record, a disputed release, a request to alter a record — involve the hospital's legal adviser before acting, and record that you did.

0.7 Course contents at a glance

Deliverable	Count
Module documents	12, plus this handbook, 130 pages in all
Diagrams and figure plates	44
Applied knowledge-check questions with model answers	72, six per module
Scored module bank questions	600, fifty per module
Final assessment questions	100
Total scored questions	700, across 228 named sub-topics
Question bank files	12 module banks, 1 final assessment, 1 combined file